



## Document Management System



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**Title:** Bed Utilization in the Paediatric Wards

### Consideration for in-patient bed utilization inthe Paediatric wards

Basedon item 168 from the medical regulation issued in 2014 and in the generalinterest of work dynamics and patient care, here are some guidelines for patient admission and bed utilization

#### General guide lines

- 1) 1.All patientswho are anticipated to have long stay need to be informed early duringadmission as they will be transferred to their respective hospital once tertiary care is over.
- 2) 2.In times whenno beds are available in the wards, all elective admission need to bere-scheduled by the admitting team a day earlier. The medical record departmentwill need to be notified and inform the patients
- 3) 3. All bookedadmissions will need to be deferred or rescheduled by the parent team if thereare acutely ill patients waiting in the emergency department unless the bookedpatient is sick.
- 4) 4.Children who donot need specialized care and need only general paediatric ward admission orgeneral pediatrician supervision, need to be referred/admitted in theirrespective hospital for Omanis and to the private hospitals for expatriates.
- 5) 5.Patient presenting to the emergency department notneeding paediatric ICU or high dependency care and need primarily generalpaediatric care need to be directed to their respective hospitals and to theprivate hospitals for the expatriates.
- 6) 6.Any kind ofservice that can be provided by the private sector need to be done in privatehospitals for the expatriates.
- 7) 7. Patientswaiting for review by specialized teams or a particular test/procedure that isnot urgent and are not directly related to the current problem, need to bebooked on OPD basis instead of waiting as inpatient for longer time.

- 9) 9.Children who are discharged need **not** to be advised to present to Royal Hospital emergency department directly if getting sick again without being seen in their local hospital first.
- 10)10.Patient who are on regular long term antibiotics/specialized therapy need to have a protocol to be in their local hospitals for further admission instead of the Royal Hospital.

### Practical issues for the residents

1. 1.When writing the problem list of the patient include area of residence of patient to plan for the transfer.
2. 2.Beyond neonatal period, history of poor oral intake needs always to be assessed clinically by direct observation and guided by family concerns. Admission to the wards only if objective evidence of poor oral intake.
3. 3.Children with viral URTI who have fever related to URTI and otherwise well can be discharged provided no other risk factors including High CRP, Leucocytosis, respiratory distress or poor oral intake requiring IVF are present. Complete resolution of fever is not necessary if no other risk factors.
4. 4.Patients who can take oral antibiotics need to be commenced on oral antibiotics instead of IV except in specific indications like serious bacterial infections.
5. 5.Children who receive 4 hourly nebulization can be discharged once reduced to 6 hourly with continuation of therapy in the local health centre. They can have 6 hourly nebulization in their health centre after counseling and writing in discharge sheets.
6. 6. Parents need to be involved in children care early including suction, NGT feeding and physiotherapy to possibly shorten the duration of stay and reduce the load on paediatric high dependency.
7. 7.Transfer out of HD should be based on clinical assessment not on time duration of e.g 24 hours observation or so. Children who are well can be transferred for HD even if been there for few hours only.
8. 8.Re assessment at evening times needs to be limited to sick patients and not as a general review.

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